

Treatment plan and investigations advised

5 September 2026

For Mr M F Rahaman, to carry to every appointment. This plan has been prepared by a physician in the family from your reports of July and August 2026. Each medicine change below is to be confirmed and prescribed by your treating cardiologist — take this sheet, with the enclosed letter, to Dr Sastry at CARE Banjara Hills.

NAME Mr M F Rahaman	AGE / SEX 63 y / Male	DATE OF BIRTH 10 Feb 1963	PLACE Hyderabad
WEIGHT 81 kg	HEIGHT _____ cm — to measure	LAST BP 150 / 100 mmHg	DIABETES reported none — HbA1c pending

DIAGNOSIS What the reports show

1.

Atrial cardiomyopathy with heart failure, preserved ejection fraction (HFpEF)

Cardiac MRI, CARE Banjara Hills, 24 Jul 2026 · EF 64% · thick, stiff left ventricle · enlarged, scarred atria
2.

Paroxysmal atrial fibrillation and atrial flutter — on a blood thinner

Diagnosed July 2026 · admission for flutter · Apollo summary 14 Aug 2026
3.

High blood pressure — not controlled at present

150/100 mmHg on current tablets · was ~120 systolic on the old combination tablet
4.

Severe obstructive sleep apnoea — not yet treated

Sleep study: AHI 42.3 events per hour (severe is above 30)
5.

Raised cholesterol — on a statin

Lipid test to be repeated
6.

Family history that matters for your medicines

Brugada syndrome (nephew, confirmed) · brothers with fainting episodes · heart attacks around age 60 (grandfather, uncle)

Rx Medicines M · A · N = MORNING · AFTERNOON · NIGHT

#	MEDICINE	DOSE	M	A	N	HOW TO TAKE	STATUS
CONTINUE AS YOU ARE TAKING NOW							
1	Tab. Metoprolol succinate MET-XL	25 mg	1 M	0 A	0 N	After breakfast. Do not increase this tablet for blood pressure — it is for heart rhythm only.	CONTINUE
2	Tab. Apixaban Eliquis	5 mg	1 M	0 A	1 N	Blood thinner. Never miss a dose. Tell any doctor or dentist you take it.	CONTINUE
3	Tab. Atorvastatin Storvas	10 mg	0 M	0 A	1 N	At night. Dose to be reviewed once the lipid test (Investigation 4) is done.	CONTINUE
ASK THE CARDIOLOGIST TO CHANGE							
4	Tab. Perindopril Coversyl	2 → 4 mg, then 8 mg	0 M	0 A	1 N	At night, one tablet. The doctor will raise the strength to 4 mg, and after 2–4 weeks to 8 mg if BP is still high and the kidney test is normal — always one tablet of the new strength; do not change it yourself. If a dry cough starts, tell the doctor — Coversyl can be swapped for Telmisartan 40 mg (the “40” in your old tablet). Never take Coversyl and Telmisartan together , and do not restart the old combination tablet while on Coversyl.	INCREASE
5	Tab. Amlodipine separate 5 mg tablet	5 mg	1 M	0 A	0 N	Morning, as a tablet on its own — not your old combination tablet , which also contains telmisartan. This is the “5” that kept your BP at 120, and it is safe with atrial fibrillation. If your ankles swell, tell the doctor — the dose can be halved.	RESTART
ASK THE CARDIOLOGIST TO ADD – 2–4 WEEKS AFTER THE CHANGES ABOVE, ONCE THE BLOOD TESTS ARE BACK							
6	Tab. Spironolactone	25 mg	1 M	0 A	0 N	For the blood pressure, and to protect the heart’s upper chambers. Morning. Start only when Dr Sastry has seen the potassium and kidney results (Investigation 1) and the aldosterone test (Investigation 2) has been drawn — he will tell you when. Repeat the potassium and kidney test 1 and 4 weeks after starting.	START AFTER TESTS

#	MEDICINE	DOSE	M	·	A	·	N	HOW TO TAKE	STATUS
7	Tab. Dapagliflozin or Empagliflozin 10 mg	10 mg	1		0		0	Morning. For the heart failure — works whether or not you are diabetic. You will pass more urine at first; drink normally.	START
8	Auto-PAP machine CPAP for sleep apnoea	nightly	≥ 4 h		aim	6+		The most important item on this page. Steadies the heart rhythm, protects the heart, and improves your sleep and breathlessness; the blood pressure benefits too, more slowly. Use it every night. A rental trial (about ₹2,000) can start this week. Bring the machine's card to your reviews.	START NOW

ONLY IF A TEST SHOWS YOU QUALIFY

9	Tirzepatide weekly injection	—	—			Only if your height (Investigation 11) gives a BMI of 30 or more and the HbA1c (Investigation 3) shows you are not diabetic. Treats sleep apnoea, heart failure and weight together. In addition to the machine, never instead of it.	TO DISCUSS
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DO NOT TAKE — SHOW THIS TO EVERY DOCTOR

AVOID

Flecainide · Propafenone · any “pill in the pocket” rhythm tablet

Because of the Brugada syndrome in our family, these heart-rhythm tablets — and any other tablet of the same family — can be dangerous for you until a heart-rhythm specialist (electrophysiologist) has assessed you, starting with the special ECG (Investigation 10).

INVESTIGATIONS ADVISED Tests advised

Blood tests 1–7 can be done at any laboratory before the visit; Dr Sastry will order the rest. Do not delay the appointment or the machine to finish them.

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|---|---|
| <div>☐ 1 Serum potassium, creatinine, eGFR
Before spironolactone — and again 1 and 4 weeks after starting it</div> <div>☐ 2 Aldosterone–renin ratio
Must be drawn before spironolactone is started</div> <div>☐ 3 HbA1c and fasting glucose
Confirms you are not diabetic</div> <div>☐ 4 Lipid profile, fasting
Decides whether 10 mg of atorvastatin is enough</div> | <div>☐ 5 TSH (thyroid)
Thyroid trouble can cause atrial fibrillation</div> <div>☐ 6 NT-proBNP
Heart-strain blood test</div> <div>☐ 7 Serum ferritin and transferrin saturation
Iron — low iron worsens breathlessness and is treatable</div> <div>☐ 8 24-hour ambulatory blood pressure monitor
Confirms the readings and checks your BP at night</div> |
|---|---|

☐ 9 **Holter / extended heart-rhythm monitor**
Shows how much of the day you are in AF, and whether the rate is controlled when you walk

☐ 10 **12-lead ECG with high leads**
Ask for "V1-V2 at the 2nd and 3rd intercostal space" – the Brugada check

☐ 11 **Height, measured on a stadiometer**
Not in any report – decides your BMI and whether medicine 9 is an option

☐ 12 **Repeat echocardiogram once the heart rate is steady**
As the 23 Jul 2026 echo report itself recommended

APPOINTMENTS Who to see

→ **Dr B. K. S. Sastry** — Heart Failure, CARE Hospitals, Banjara Hills
Take the enclosed letter. He will confirm the medicine changes above.

→ **Dr Soumen Devi Dutta** — Electrophysiology, Apollo Hospitals, Jubilee Hills
To discuss a procedure for the flutter (cavotricuspid isthmus ablation), and rhythm control for the AF — the best window for that closes around July 2027.

→ **Sleep clinic / PAP supplier**
Machine set-up and mask fitting this week; pressure check and usage review at 4 weeks and 12 weeks.

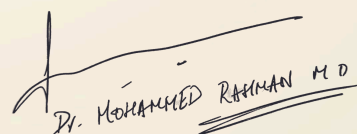
ADVICE Every day

- **Blood pressure** morning and evening, seated, after 5 minutes' rest. Write it in a diary and bring the diary.
- **Weight** every morning. Tell the doctor if it rises by 2 kg or more in a week.
- **Ankle swelling** — note it and mention it. Do not stop any tablet on your own.
- **Salt low; alcohol none.** Keep walking daily — to the distance that is comfortable, not beyond.
- **Go to hospital at once** for fainting or near-fainting, palpitations with dizziness, chest pain, or breathlessness at rest.
- Bring **this sheet and all reports** to every visit.

REVIEW When to check

- **2 weeks** after any tablet change — BP check.
- **1-2 weeks** after each perindopril increase — kidney and potassium test.
- **1 week and 4 weeks** after starting spironolactone — kidney and potassium test.
- **4 weeks and 12 weeks** after starting the machine — usage and BP review.
- **Cardiology follow-up** as Dr Sastry advises.

Prepared with love, and with your reports in front of me.
Nothing here replaces your cardiologist's judgement — it is written so that the two of you start the conversation from the same page.



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